



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : غاده ابراهيم علي عبد السلام

المحافظة : الجيزة الحي / المدينة : الهرم

رقم البطاقة : 29203012105204 رقم التليفون : 01121304091

العمر (تاريخ الميلاد) : 01-03-1992 المهنة : ربه منزل

العنوان : قرية القيراطين

تاريخ تسجيل الإستمارة : 19-09-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: غاده ابراهيم علي عبد السلام
National ID: 29203012105204



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Bilateral mammogram and breast ultrasound

الوقت :

-- --:--

التاريخ :

05-07-2026

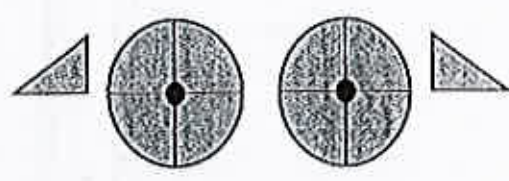
الطبيب المعالج :
د/شيماء لاشين



MDT FORM 13/11/2025 G

Name	غادة ابراهيم علي	MRN	1011/2025
Age	33 y	ID	292030121059044
Menopausal status	pre	Phone	01121304091
NRS SCORE		Marital status	married
BMI		Family history	-ve
CO morbidity	-ve	REASON OF MDT	post op

Clinical Data

BX:	IDC			R	L
ER:	818	PR:	318	HER 2:	+/ve
Sonomammography					
RT breast LQ Cluster of microcalcification				TNM	
Mets Workup:					
CT → free					
Bone scan → free					

Recommendations:

Post NACI R+BES+SLNB
No residual viable remnants
No DCIS of atypical ductal
Multiple foci of atypical ductal
SLNB (014)
To No

مريم عبد الله

MDT OUTCOME & PLAN

PORT
Continue phesgo one year.
→ Start TMT after PORT (+) Total

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/زهره	د/احمد	د/احمد			

Patient Name : الأستاذة / غاده ابراهيم على عبد السلام
Patient No. : 29203012105204
Path No. : 3157/25
Sex : Female
Age : 33 Y
Referring Doc :

Request No. : 13006612
Request Date : 15/10/2025
Verification Date : 21/10/2025
Financial Category : جوستاف روسي - نفقة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer, Post-NACT.

Nature of specimen: Right breast conservative surgery & sentinel lymph nodes biopsy.

Gross Examination:

- * Right breast conservative surgery specimen measured 10x08x04 cm, with covering skin ellipse measured 09x03 cm.
- * Serialing showed wire guided clip inserted tumor bed site measured 3.8x2.1x1.3 cm, located 2.5 cm from medial margin, 2 cm from lateral margin, 0.7 cm superior margin, 1 cm from (inferior margin & skin) and 3.5 cm from deep margin.
- * Sentinel lymph nodes: Fibrofatty tissue piece measured 4.5x2.5x1 cm, entangling 4 lymph nodes, the largest was 1.5x1x0.5 cm.
- * 12 Blocks:
1:4- T.B.S., 5- Medial margin., 6- Lateral margin., 7- Superior margin., 8- Inferior margin., 9- Deep margin., 10- Retro-areolar., 11- Skin., 12- SLNs.
- Gross was done by; Dr. Heba Hassan.

* Frozen (Intra-operative) Results:

- Free surgical margins.
- Free four sentinel lymph nodes (0/4).

Microscopic Examination:

- * Histological type: No residual viable remnants of the previously diagnosed tumor.
- Cellularity (0%).
- * Angioinvasion: Absent.
- * Ductal carcinoma in situ: Absent.
- * Additional findings: Multiple foci of atypical ductal hyperplasia & Effect of therapy with microcalcification.
- * Surgical margins: Free.

Verified by :

Dr.Duaa Mohamed

Printing Date : 28/10/2025 09:38:53

Page 1 of 2

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / غاده ابراهيم على عبد السلام
Patient No. : 29203012105204
Path No. : 3157/25
Sex : Female
Age : 33 Y
Referring Doc :

Request No. : 13006612
Request Date : 15/10/2025
Verification Date : 21/10/2025
Financial Category : جوسفات روسي - نفقة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

- * Sentinel lymph nodes: Four free lymph nodes, LNs (0/4).
- * Histologic response to therapy: Pathological complete response to therapy.

Diagnosis:

Right breast cancer (Post-NACT), Right breast conservative surgery & sentinel lymph nodes biopsy:

- No residual viable remnants of the previously diagnosed tumor.
- No ductal carcinoma insitu (DCIS).
- Multiple foci of atypical ductal hyperplasia.
- Surgical margins: Free.
- Sentinel lymph nodes: Free 4 lymph nodes, LNs (0/4).
- Histologic response to therapy: Pathological complete response to therapy.
- Stage: ypT0, ypN0 (sln).

Signature

Dr. Walaa Ibrahim Ghoneim.

Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

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Page 2 of 2

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذة / غادة إبراهيم على عبد السلام	Request No.	: 13004600
Patient No.	: 29203012105204	Request Date	: 06/04/2025
Path No.	: SR-517/25	Verification Date	: 14/04/2025
Sex	: Female	Financial Category	: أورام دار العلام - فئة الدولة
Age	: 33 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Right breast upper inner quadrant mass.
Nature of Specimen: Slide Revision.

Gross Examination:

- Two paraffin blocks labeled by (200278/25) and five slides were received.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade III (3+3+2).
- * DCIS: DCIS (solid pattern).
- * Angiolymphatic invasion: Not detected.
- * Perineural invasion: Not detected.
- * Microcalcifications: Positive.
- * ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- * PR: Faint nuclear reaction in 10% of tumor cells (score 3/8).
- * Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).

Diagnosis:

Right breast upper inner quadrant mass, Slide revision:
Invasive duct carcinoma, grade III, with DCIS (solid pattern) and microcalcifications.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 3/8).
- Her2: Positive (score 3+).

Verified by :

Dr. Toka Medhat

Printing Date : 03/05/2025 09:59:55

Page 1 of 2

Printing By : marwa mohamed yousif



غرفة رقم (4) - الدور الأول - مبنى رقم (287) - جسر النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 287 - Nile Corniche - Mir El Qadima - Cairo, Egypt

PATIENT NAME: GHADA IBRAHIM ALI ABDEL SALAM

GENDER: Female

DATE : 10-Mar-2025 02:13 PM

PATIENT ID: 20225017185

AGE: 33 y

ORDER: Z171456,Z171457

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- Right 3 o'clock lump.

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

Right side:

- Heterogeneously dense breast parenchyma (ACR C).
- Right LIQ ill defined asymetry with distortion and overlying suspicious clustered microcalcifications, corresponds to U/S detected right 3-4 o'clock ill defined area of altered hypoechoic parenchyma with underlying irregular supicious masses/nodules and microcalcifications with increased vascularity. Distance from the nipple is 6 cm.
- Prominent fibroglandular parenchyma with mottling denoting adenosis.
- Right axillary level I mildly enlarged lymph node with diffuse increased cortical thickness ? indeterminate node.
- No pathological right infra or supraclavicular lymph nodes.

Left side:

- Heterogeneously dense breast parenchyma (ACR C).No spiculated mass lesions or grouped microcalcifications.
- This was confirmed by Complementary Ultrasound which showed no solid or cystic mass lesions.
- Prominent fibroglandular parenchyma with mottling denoting adenosis.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Nonspecific axillary lymph nodes.

OPINION:

- Right 3-4 o'clock highly suspicious malignant process (BIRADS 4C) with ipsilateral indeterminate axillary level I lymph node for histopathological confirmation.
- Bilateral mild fibroadenosis (BIRADS 2)

RECOMMENDATION:

Referral to clinic





Patient Name: Ghada Ibrahim Ali Abd Al Salam	Date : 13/05/2025
Exam : C.T CHEST	Age : 33 Year(9)
Medical ID: 1011/2025	Pat. ID: 99510

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- Right breast UIQ Irregular lesions the largest is measuring 3.8 x 4 cm.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitory lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. No definite enhancing focal lesions or contour irregularities.
- No Intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- IUD is seen.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

BQHEYA

Radiology Department

بهاية
مركز الأشعة التشخيصية

PATIENT NAME: GHADA IBRAHIM ALI ABDEL SALAM
GENDER: Female
DATE : 18-Mar-2025 10:58 AM

PATIENT ID: 20225017185
AGE: 33 y
ORDER: 3295367,3295368

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN
AND PELVIS WITH TRIPHASIC STUDY OF THE LIVER**

TECHNIQUE:

- Multislice helical CT acquisition with automatic IV low osmolar non-ionic contrast medium infusion.

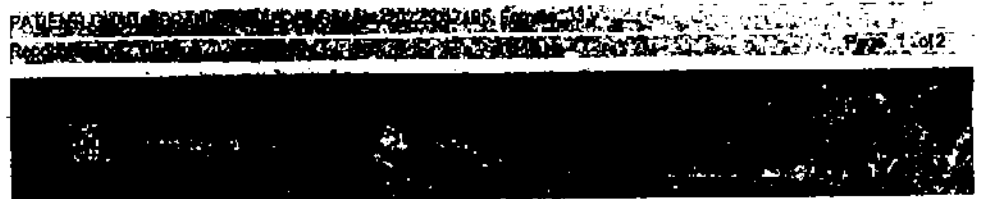
FINDINGS:

1-CHEST

- Right breast LIQ spiculated heterogeneously enhancing mass lesion is seen measuring about 4.7 x 4.1 cm along its maximum axial dimensions and showing surrounding nodular lesions and parenchymal heterogeneity.
- Few prominent right axillary level I lymph nodes, the largest measures about 13 x 12 mm along its maximum axial dimensions.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No CT evidence of pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.

2- ABDOMEN AND PELVIS

- The liver is mildly enlarged showing homogenous CT texture. No focal lesions or intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.



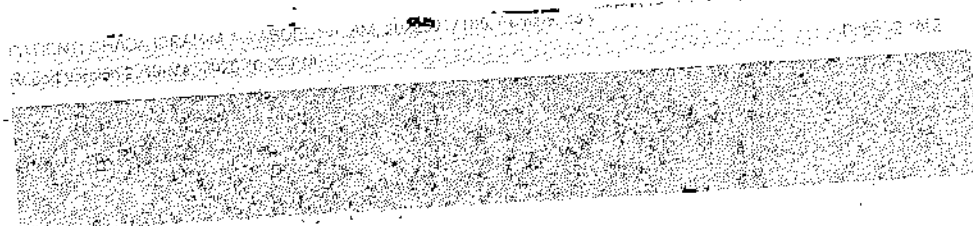
- Normal appearance of the pancreas, spleen, kidneys, adrenals, aorta, l. V.C. and the pelvic organs.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
 - No pathologically enlarged abdominal or pelvic lymph nodes.
 - No abdominal or pelvic masses or collections.
 - **The revised bone window revealed:**
 1. Few sclerotic foci seen implicating some vertebral bodies, pelvic bones and femoral heads.
 2. Small sternal lucent area measures about 7 mm.
 3. No CT evidence of vertebral collapse or extra-osseous soft tissue component.
 4. Mild osteitis pubis.

Opinion:

- Right breast neoplastic process as described.
- Few osseus sclerotic foci and small sternal lucent area, for bone scan.
- No sizable pulmonary or hepatic metastatic deposits.

Reported By,
Dr. Sara Saif, MSc
Dr. Mennatallah Mohamed, MD

19-Mar-2025 10:09 AM



ECHO REPORT

NAME :

كاهلارام باب

DATE: 18/1/2026

ID NUMBER: 1011 / 2025

SEX: ♀ AGE: 33

EF	62%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.8	3.5-5.8cm	AO.R	2.3	3-3.7cm
LVEDD	2.6	2-4cm	LA	3.6	2-4cm
FS	29%	25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions *Normal*

LV Wall Thickness *Normal*

LV Diastolic Function *Normal*

SWMA *No RWA*

LA and Ao Root Dimensions *Normal*

Mitral Valve

Aortic Valve

Tricuspid Valve

Pulmonary Valve

Right Side and Systolic Function

Intracardiac Masses and Effusion

DR.

TAPSE: 2.3